

Patient 19**Report A**

6553579 05/07/2016 CT Acute Stroke

For Thrombolysis? yes

Report:

Time 1st called: 00:30

Time plain CT available on PACS: 00:30

Time of verbal report to stroke registrar:00:40

Indication: L-sided stroke

CT HEAD

Haemorrhage: no Intravascular thrombus: yes, there is hyperdensity within the tip of the ICA, extending to the right M1 in keeping with an acute intravascular thrombus. Acute infarct: yes, there is loss of the grey-white matter differentiation and sulcal effacement involving the right insula, frontoparietal and temporal operculum, and the right lentiform nucleus is indistinct in keeping with acute infarction in the right MCA territory, Other findings and comments: There are shallow subdural hygromas overlying both cerebral convexities, measuring 10mm in depth on the left side, and 5mm on the right side. There is no significant mass effect on the underlying brain parenchyma.

CT ANGIOGRAM

Window settings: (intraluminal HU=270; W=540; L=200)

ICA Stenoses NASCET method, measurements on axial 1mm sections)

Aortic arch and subclavian arteries: Calcified plaques within the aortic arch do not cause significant stenosis.

RCCA/ICA: Calcified plaques within the cavernous and supraclinoid segments of the ICA do not cause significant stenosis.

LCCA/ICA: Calcified plaques within the cavernous and supraclinoid segments of the ICA do not cause significant stenosis.

RVA. The VA enhances normally with no focal stenosis.

LVA: The VA enhances normally with no focal stenosis. Intracranial Arteries: There is a filling defect within the tip of the right ICA which extends to the right M1, with hardly any enhancement seen within the vascular territory of the right MCA.

Other findings and comments:

Mild spondylotic changes are noted within the cervical spine.

Conclusion: There is an acute thrombus within the right ICA tip extending to the right M1, with evidence of acute infarction in the right MCA territory. DNAD

Report B

6554516 05/07/2016 CT head

Clinical indications:

24 hours post thrombolysis

Findings:

Comparison is made with the CT study performed today at 00:21

There is now conspicuous low attenuation involving the right frontal, temporal and parietal lobes, insula and basal ganglia, in keeping with an evolving acute right MCA territory infarct. There is associated local mass effect, but no midline shift is currently demonstrated. No haemorrhage transformation is shown. The remainder of intracranial appearances are stable. Note is made of persistent hyperdensity within the right terminal ICA and proximal MCA.